

Riverside Lymphology & Vascular Outpatient Clinic

Department of Lymphology · 14 Mill Lane, Northbridge · Tel. 0000 000 000

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Patient ID	TEST-4471	Date of assessment	14 March 2026
Age / sex	42, female	Referred by	General practitioner
Seen by	Dr. A. Meyer, Lymphology	Report type	Initial assessment

Assessment of suspected lipoedema of the legs and arms

Reason for referral

Progressive, symmetrical enlargement of both legs over approximately eight years, with pain that is out of proportion to the swelling and marked tenderness on light pressure. Referred for assessment and for advice on conservative treatment.

History

The patient reports that the shape of both legs began to change in her mid-twenties and has changed further after each of two pregnancies. She describes both legs as heavy and tired, most noticeably towards the end of the day and in warm weather. Standing for longer than twenty minutes is uncomfortable. She bruises very easily, often without recalling any knock. Two courses of dieting over the last five years reduced her waist but made no difference to the legs or the upper arms.

She sleeps poorly, wakes several times a night with cramp, and is tired throughout the day. Her mother and a maternal aunt have a similar leg shape. Weight has been stable for three years. She is otherwise well, takes levothyroxine for hypothyroidism, and has no history of deep vein thrombosis, cellulitis or heart failure.

Examination

Both thighs show pronounced, symmetrical fat deposits over the front and outer aspect, with a firm, nodular texture on palpation that is most marked on the outer thigh on both sides. A cuff-like fat pad sits just above both ankles; the feet themselves are entirely spared, with no swelling of the toes and a negative Stemmer sign bilaterally. The skin of both calves has a coarse, dimpled surface. Fine whitish stretch marks run along the inner aspect of both thighs. There is a bluish-purple discolouration over the left shin, consistent with the easy bruising described above.

By the end of the day both lower legs show soft pitting swelling, more pronounced on the left. Both upper arms show softer, symmetrical fat deposits on their back aspect, tender to pressure; the forearms and hands are unaffected.

On questioning about pain, she reports a deep, aching pain across both thighs that is present most days, and a sharp, shooting pain in the left calf that is reproduced on palpation. She describes a burning sensation over the front of both lower legs in the evenings. The skin over both knees is painful to light touch. Both calves cramp at night.

Findings unrelated to the lipoedema: a well-healed surgical scar in the right lower abdomen (appendicectomy, 2009), not tender. An itchy, reddened rash in both armpits, present for about three weeks, most likely intertrigo. Morning stiffness and pain in the right wrist for the past few months, with no swelling.

Assessment

Lipoedema, stage 2, involving both legs and both upper arms, with secondary lymphatic congestion of the lower legs. Intertrigo of both armpits. Suspected early osteoarthritis of the right wrist, for which a separate referral has been made. Fatigue and disturbed sleep are likely multifactorial and should be reviewed alongside her thyroid function.

Plan

- Flat-knit compression garments for both legs, class 2, to be worn daily; measured this week.
- Manual lymphatic drainage twice weekly for six weeks, then reassess.
- Intermittent pneumatic compression at home in the evenings.

- Skin care for the armpit rash; topical treatment prescribed and wound care advice given.
- Physical therapy with a focus on the legs; water-based exercise encouraged three times a week.
- Liposuction was discussed as an option should conservative treatment prove insufficient. No decision has been taken and no date has been set.
- Review in the outpatient clinic in three months.

Circumference measurements (cm)

Taken standing, in the morning, at fixed distances from the floor. Baseline for comparison at review.

Level	Right	Left	Difference
Ankle, above the malleoli	24.5	25.8	1.3
Calf, widest point	41.0	42.6	1.6
Below the knee	38.2	39.1	0.9
Thigh, 10 cm above the knee	58.4	59.0	0.6
Thigh, widest point	68.9	69.7	0.8
Upper arm, widest point	36.1	36.4	0.3

The consistent left-sided excess is in keeping with the more pronounced swelling of the left lower leg described above.

Impact reported by the patient

She avoids skirts and dresses and finds shopping for trousers distressing. She has stopped attending an exercise class because of the pain in her thighs afterwards. She has been told several times that the problem would resolve if she lost weight, which she experiences as dismissive. She would like a clear explanation to give to her family, and information on whether her daughters are likely to be affected.

Follow-up

Review in three months with repeat circumference measurements and photographs. Sooner if the skin over the shins breaks down, if the armpit rash fails to settle within two weeks, or if either leg becomes red, hot and painful.